



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : ايمان محمد محمود سليم
المحافظة : القليوبية
الحى / المدينة : الخصوص
رقم البطاقة : 27605010101744 رقم التليفون : 01122886191
العنوان : سعيد
العمر (تاريخ الميلاد) : 01-05-1976 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ايمان محمد محمود سليم National ID: 27605010101744	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ايمن
محمد محمود سليم
هش شكري سعيد
الخصوص - القنوبية

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حاصله على دبلوم صنايع

أنثى مسلمة

البطاقة سارية حتى ٢٠٢٣/٠١/٠٤

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Patient Name : الأستاذة / ايمان محمد محمود سليم	Request No. : 13005623
Patient No. : 27605010101744	Request Date : 09/04/2025
Path No. : 1246/25	Verification Date : 29/04/2025
Sex : Female	Financial Category : اورام دار السلام - ميلد
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Right wide local excision and axillary clearance specimens , collected at 9/4/2025.

Gross Examination:

Five specimens were recieved:

- Axillary clearance specimen : Two Fibrofatty tissue peices measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm .
- Extra medial margin:Fibrofatty tissue piece measured 3x2.5 cm
- Extra deep margin: Fibrofatty tissue piece measured 2.5x2 cm
- Right wide local excision specimen measured: 10x8x4 cm with covering skin ellipse measured 9x3.5 cm.
- Tumor extent
 - o Number: Two (wired) ill defined rubbery greyish white areas .
 - o Size: 1.5x1x1cm and 1x0.5 cm.
 - o Skin involvement: Free grossly
- Distance from margins: 0.5 cm from least lateral margin.
- Frozen section:
 - Lymph nodes : Free (0/6), six more nodes are left for paraffin examination.
 - Suspicious medial and deep margins
 - Free other margins.

Microscopic examination:

- First wired area:
 - Histological type:Invasive duct carcinoma.
 - Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).

Verified by :

Dr.Heba Hasan

Name: إيمان محمد محمود MR. : 2897	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمناظرة وجد انه The patient is a premenopausal individual with right-sided breast cancer, clinical stage T2, node-negative (N0), estrogen receptor (ER) score 8/8, progesterone receptor (PR) score 5/8, and HER2 3+. Neoadjuvant chemotherapy with the TCHP regimen was administered, followed by surgery on 9 April 2025 (breast-conserving surgery). Pathology revealed ypT1 ypN0. Adjuvant endocrine therapy with an aromatase inhibitor (AI) was started in May 2025, and the patient began trastuzumab-emtansine (KADCYLA) in July 2025. Baseline hormonal studies showed elevated estradiol (E2) and follicle-stimulating hormone (FSH). An echocardiogram performed on 29 December 2025 demonstrated a left ventricular ejection fraction (EF) of 64%. The treatment plan at that time included continuation of KADCYLA, addition of letrozole (FEMARA), and pending initiation of zoledronic acid (ZOMETA) pending dental clearance. The patient was scheduled for a bone scan in January 2026, which was subsequently reported as free of disease. Subsequent follow-up on 8 February 2026 reiterated the EF of 64% and confirmed the ongoing regimen of KADCYLA and FEMARA, with ZOMETA still pending dental evaluation. The plan emphasized continuation of the current therapy pending dental clearance. On 2 March 2026, the same EF of 64% was documented, and the plan again included KADCYLA and FEMARA, with a dental examination required before initiating ZOMETA. Pharmacy orders were prepared accordingly. A pharmacy note on 23 March 2026 reiterated the EF of 64% and indicated that the next cycle of KADCYLA (C11) and FEMARA would proceed, with ZOMETA pending dental approval. An echocardiogram on 29 March 2026 showed an improved EF of 66% and confirmed that laboratory parameters were adequate. The patient was to receive cycle 11 of KADCYLA together with FEMARA and supportive therapy. On 30 April 2026, breast ultrasound performed in January 2026 was reported as disease-free. Hormonal studies showed elevated estradiol (E2) and FSH, and a repeat assessment was planned for February 2026. The patient was scheduled for cycle 12 of KADCYLA at a dose of 240 mg (weight 67 kg) together with FEMARA, with ZOMETA pending dental clearance. The patient has now completed twelve cycles of KADCYLA.	
تحريراً في ,	
التاريخ :	19-09-2026
الطبيب المعالج :	dina makram
يعتمد ,	

Patient Name : الأستاذة / ايمان محمد محمود سليم
Patient No. : 27605010101744
Path No. : 1246/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Tumor cellularity: 20%.
- Angioinvasion: Not detected.
- Ductal carcinoma in situ: Not detected
- Second wired area :
- Only ductal carcinoma component, intermediate grade, (5%).
- There is no evidence of invasive component
- Lymph node involvement: Positive (1/12), with some showed therapy effect.
- Frozen nodes (1/6).
- Paraffin nodes (0/6).
- Resection Margins: Free all resected margins and extra margins.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, wide local excision and axillary clearance specimens:

- Invasive duct carcinoma , grade II.
- Tumor cellularity: 20%.
- Free all resected margins and extra margins.
- Lymph nodes: Positive (1/12).
- Pathologic partial response.
- Stage : ypT1b , ypN1.
- RCB : 2.767 , RCB class II

Comment: There is a disparity between frozen and paraffin results, however final diagnosis depends only on paraffin results.

Prof. Gamal Abdelatty
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 2 of 3

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atient Name	: الأستاذ / سميح بيومي عبد الله محمد	Request No.	: 13002998
atient No.	: 27807092101901	Request Date	: 05/12/2024
ath No.	: 4780/24	Verification Date	: 15/12/2024
ex	: Female	Financial Category	: أورام دار السلام - ميانره
ge	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 5/12/2024.

Gross Examination:
- 4 grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Focal.
* Perineural invasion: No.
* Angioinvasion: No.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with focal DCIS.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

Printing Date : 08/04/2025 12:57:08

Page 1 of 1

Printing By : hermal





Name: Mrs. Eman Mohamed Mahmoud Selim

Date: 9/11/2024

ID: 2224069566

Acc: 5047373

Referring Hospital Awram Dar Al-Safaani.

Te-99m MDP whole body bone scan

Technique:

Whole body images were acquired 180 minutes after IV injection of 15 mCi of the tracer.

Findings:

Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.

Degenerative changes of the spine are seen.

Arthritic knee changes are noted.

Both kidneys are visualized.

Opinion:

No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandeel, MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by AR





Patient Name: Eman Mohamed Mahmoud Selim	Date : 25/11/2024
Exam : C.T NECK	Age : 48 Year(s)
Medical ID: 3087/2024	Pat. ID: 77240

NON CONTRAST MULTI-SLICE CT SCAN OF THE NECK: -

- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilaginous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal appearance of the neck muscles & vessels.
- Multiple small deep cervical lymph nodes noted, being subcentimetric in short axis.

Opinion:

- Multiple small deep cervical lymph nodes, likely inflammatory than neoplastic for clinical correlation & follow up.

Dr. Sherif Sameh, M.D.



Ministry of Health and Family Welfare
Government of India

Ministry of Health and Family Welfare
Government of India



MDT form

1. Patient ID

2. Referral

3. Referral Date

4. Referral

5. Medical History

6. Referral

7. Referral

8. Referral

9. Referral

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30. Referral

Clinical data

Breast Imaging :

at breast UOQ multiple speculated irregular mass lesion from 10 to 15 cm
- 12.2 cm from skin, nearest one from nipple 20 mm from post
and 14 x 12 BIRADS 5
- on RT indeterminate primary LN

Metastatic Workup :

None

BR

ER

PR

HER 2 Neu

SIEN

K167

10C

B/B

S/B

2

4

1

N

M

2

0

0

Sonohemimammography

CESM: UOQ INDEX LESION 24MM, 2 OTHER LESIONS, IPSILATERAL PATH AXILLA

PATIENT HISTORY

received NACT TCHP x 6 cycles

RT BCS+AD: free margin

YPT 1m largest diameter (1.5CM) LN 1/12

partial pathological response

MDT OUTCOME & PLAN

adjuvant Edoxyla x12

part

adjuvant AI

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

Psychiatric

RADIOLOGY

PATHOLOGY

nutrition



Patient Name: Eman Mohamed Mahmoud Selim	Date : 24/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 3087/2024	Pat. ID: 77240

**COMBINED BILATERAL BREAST MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND OF BOTH BREASTS**

Indication:

Multifocal right breast pathologically proven malignant masses.

Received NACT

Report:

- Scattered fibro glandular density (ACR B)
- Stable right breast show UOQ multiple spiculated irregular mass lesion, of high density. The area involving the masses measures about 66x65x110mm. Two of which are marked by metallic clip.
- Complementary ultrasound revealed right breast multiple irregular spiculated hypoechoic mass lesions are seen from 10 o'clock to 12 o'clock, 2mm from skin and nearest one from nipple about 36mm, the largest one measuring about 15x10mm .
- No left suspicious micro-calcifications or spiculated masses.
- No left solid or cystic mass lesion.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Still seen one indeterminate right axillary lymph nodes are seen with diffuse cortical thickening about 3.3mm and preserved fatty hilum.
- Nonspecific left axillary LN.

Opinion:

- Right breast UOQ pathologically proven malignant masses with still noted ipsilateral indeterminate axillary lymph node showing overall stationary course (BIRADS 6).
- Normal study of the left breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Patient ID: 00519	Patient Name: English: Eman Mohamed Mahmoud Sellin
Ref: 11-2025-000000	0025/04/09 10:47:51 AM Report Date

Patient Name: Eman Mohamed Mahmoud Sellin

Date: 09/04/2025

Stereotactic Guided Wire Localization:

Clinical summary: Status post NACT for pre-operative localization of inserted clips.

Technique:

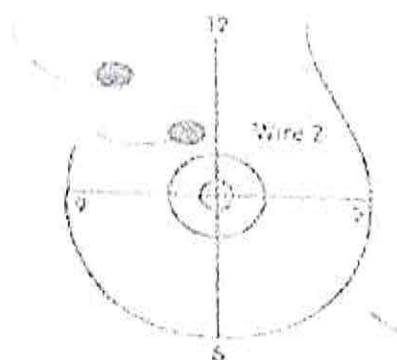
Under complete sterile conditions, 3ml of local anesthetic were administered at the site of concern in the RT breast. Two 21G wires were used to localize the inserted clips.

The wires are located at the right upper outer quadrant.

Wire 1: was inserted perpendicular to the peripherally located inserted clip.

Wire 2: was inserted perpendicular to the anteriorly located clip that was at 5.7 cm from the nipple with the hook seen anterior to the clip by 3.5mm.

Wire 1



Much Obliged
Dr. Sherihan Fakhry, MD.



جمهورية مصر العربية
وزارة الداخلية
إدارة الأحوال المدنية

ايمان
محمد محمود سليم
ش. شكرى سعيد
الخصوص - القليوبية



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حاصله على دبلوم صنایع

مسلمة

أنشئ



البطاقة سارية حتى ٢٠١٧/٠٩/٢٢

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